

Tepezza Literature Surveillance

PubMed case reports and case series - integrated title, abstract, and full-text surveillance report

98	Broad records	19	Included	7	Public full text
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Scope

This report documents a reproducible PubMed search for patient-level Tepezza/teprotumumab case reports and case series published in the prior year. It records title/abstract screening for 19 included publications and completed medical review of the seven publicly accessible full texts. The full-text section assesses minimum ICSR criteria, seriousness, expectedness, causality, dechallenge/rechallenge, confounders, duplicate considerations, aggregate relevance, and final routing.

Database	PubMed
Product terms	Teprotumumab; teprotumumab-trbw; Tepezza; HZN-001; R1507; RG1507
Date window	12 Sep 2025 through 12 Sep 2026; PubMed Date - Publication
Run date	12 Sep 2026
Broad retrieval	98 unique PubMed records
Case-report/series candidates	21 records
Included after title/abstract screen	19 patient-level reports or series
Public full text	7 available; 12 not publicly accessible at run date
Review/QC	Initial evaluation by Rita Rui-Ci Lin, PhD, reconciled with AI-assisted full-text review; independent second reviewer pending

Prepared for: Rita Rui-Ci Lin, PhD | Search and screening date: 12 September 2026

Search strategy and screening rules

The broad search used all indexed fields for the proprietary name, proper/generic name, and development identifiers. A second query applied publication-type and title/abstract case-report or case-series terms. All 98 broad-search titles and abstracts were considered; 21 candidate records received detailed abstract screening.

Inclusion: an original patient-level case report or case series in which teprotumumab was administered, suspected, or materially part of the clinical history. Retrospective publications explicitly described as case series were retained. Exclusion: reviews without new cases, database disproportionality studies, trials/cohorts not described as case reports/series, editorials, and reports in which teprotumumab was only mentioned as a future option.

Full-text available means publicly accessible HTML or PDF through PubMed/PMC or the linked publisher on 12 September 2026. Institutional-subscription access was not counted. A DOI landing page or abstract alone was not counted as public full text.

Date convention

The surveillance window uses PubMed's Date - Publication field. Records marked with an asterisk have an issue date inside the window but an online-first date before 12 September 2025. They remain included for reproducibility because PubMed retrieved them, but a strict online-first-date analysis would exclude PMIDs 40396469, 40488249, and 40876911.

Exact broad PubMed query

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(teprotumumab[All Fields] OR "teprotumumab-trbw"[All Fields] OR Tepezza[All Fields] OR "HZN-001"[All Fields] OR R1507[All Fields] OR RG1507[All Fields]) AND ("2025/09/12"[Date - Publication] : "2026/09/12"[Date - Publication])
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Exact case-candidate addition

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AND ("Case Reports"[Publication Type] OR "case report"[Title/Abstract] OR "case reports"[Title/Abstract] OR "case series"[Title/Abstract])
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Full-text assessment framework

The review applies the four minimum ICSR elements in [ICH E2D\(R1\)](#): at least one adverse event/adverse reaction or other reportable observation, a suspect or interacting medicinal product, an identifiable patient, and an identifiable reporter. In literature, the first or corresponding author is the primary source, and a patient number can support identifiability. Expectedness and seriousness are evaluated separately from minimum case validity.

Expectedness was compared with the [US TEPEZZA Prescribing Information revised November 2025](#). An actual marketing authorization holder must use its controlling regional reference safety information and receipt date. ICH seriousness criteria were applied; severity is not synonymous with seriousness. Regulatory submission remains subject to regional requirements and company procedures.

List 1 - Public full text available

Seven included reports had publicly accessible full text on the search date.

PMID 42688166 2026 Aug 19	Case Report: Teprotumumab treatment in an HIV-infected patient with refractory thyroid eye disease: clinical and quantitative MRI evidence. Mingyue Zheng et al. Front Immunol. DOI: 10.3389/fimmu.2026.1911723	PMC open access
PMID 42438806 2026 Aug (online 2026 Jul 13)	Thyroid eye disease with teprotumumab-related hyperglycemia associated with increased growth hormone secretion. Ken Sugawa et al. JCEM Case Rep. DOI: 10.1210/jcemcr/luag186	PMC open access
PMID 42386595 2026 Jul 1	Teprotumumab-associated persistent unilateral hearing loss in dysthyroid optic neuropathy: a case report with review of the literature. Yuji Hiromatsu et al. Endocr J. DOI: 10.1507/endocrj.EJ26-0064	Free article - J-STAGE
PMID 41924363 2026 Apr 18	Severe nausea and vomiting after teprotumumab initiation with corticosteroid pretreatment: a case report. Justin Youn et al. Int J Ophthalmol. DOI: 10.18240/ijo.2026.04.25	PMC free full text
PMID 41487291 2026 Mar (online 2025 Dec 5)	Extraocular muscle volume reduction in a patient treated with two courses of teprotumumab for thyroid eye disease. Hanna Richkind et al. Am J Ophthalmol Case Rep. DOI: 10.1016/j.ajoc.2025.102490	PMC open access
PMID 41441522 2025 Nov 26	Case Reports of Teprotumumab as Treatment for Monoclonal Antibody-Induced Thyroid Orbitopathy. Jared Moon et al. Reports (MDPI). DOI: 10.3390/reports8040246	PMC open access
PMID 40876911 2025 Oct (online 2025 Aug 28)*	Teprotumumab Treatment in Patients with Steroid and Surgery-Resistant Dysthyroid Optic Neuropathy: A Case Series. Anna Lucia Carretti et al. Thyroid. DOI: 10.1177/10507256251372194	PubMed-designated free article

List 2 - No public full text located

Twelve included reports had an abstract/citation but no publicly accessible full text located on the search date. Library or institutional access may still be available.

PMID 42727021 2026 Sep 11	Adjunctive Orbital Radiation Therapy with Teprotumumab in Recalcitrant Thyroid Eye Disease: A Case Report. Mannat Kaur et al. Ophthalmic Plast Reconstr Surg. DOI: 10.1097/IOP.0000000000003287	No public full text located.
PMID 42702683 2026 Sep 7	Teprotumumab for thyroid eye disease-associated superior ophthalmic vein periphlebitis: a case series. Muhammad Abumanhal et al. Graefes Arch Clin Exp Ophthalmol. DOI: 10.1007/s00417-026-07491-y	No public full text located.
PMID 41718601 2026 Sep-Oct (online 2026 Feb 20)	Relapse Versus Rebound in Thyroid Eye Disease After Teprotumumab Treatment: A Retrospective Case Series. Julie M Shabto et al. Ophthalmic Plast Reconstr Surg. DOI: 10.1097/IOP.0000000000003192	No public full text located.
PMID 42475232 2026 Jul 20	Testosterone Dysregulation and Erectile Dysfunction Associated With Teprotumumab Infusions: A Case Report. Sri Meghana Konda et al. Ophthalmic Plast Reconstr Surg. DOI: 10.1097/IOP.0000000000003250	No public full text located.

PMID 41196136 2026 Jul-Aug (online 2025 Nov 6)	Transformation of Thyroid Eye Disease Phenotype Following Teprotumumab Treatment. Julie M Shabto et al. Ophthalmic Plast Reconstr Surg. DOI: 10.1097/IOP.0000000000003110	No public full text located.
PMID 41818461 2026 Jun (online 2026 Mar 12)	Recalcitrant thyroid eye disease responsive to targeted EGFR inhibition in a patient with lung adenocarcinoma: a case report. Rachel E Cain et al. Orbit. DOI: 10.1080/01676830.2026.2632696	No public full text located.
PMID 42694662 2026 May 18	Variability Between Changes in Ocular Alignment and Gorman Diplopia Score After Teprotumumab in Thyroid Eye Disease. Madhura A Tamhankar et al. Neuroophthalmology. DOI: 10.1080/01658107.2026.2673534	No public full text located. PMC embargo to 18 May 2027.
PMID 42103337 2026 May 8	From orbit to cortex: recognising teprotumumab-induced encephalopathy. Alan Spicer et al. BMJ Case Rep. DOI: 10.1136/bcr-2025-271752	No public full text located.
PMID 41700562 2026 Apr (online 2026 Feb 17)	N-acetylcysteine and Vitamin C oral antioxidant therapy for teprotumumab-related hearing dysfunction: case series and review of literature. Elijah Gregory et al. Orbit. DOI: 10.1080/01676830.2025.2608253	No public full text located.
PMID 40396469 2026 Apr (online 2025 May 21)*	Teprotumumab Use in Thyroid Eye Disease: Clinical Outcomes in the United Arab Emirates- a First Regional Case Series. H Eatamadi et al. Semin Ophthalmol. DOI: 10.1080/08820538.2025.2503911	No public full text located.
PMID 41791882 2026 Feb (online 2026 Jan 13)	Beyond the Eyes: Is Teprotumumab Effective for Thyroid Dermopathy? Dana Hamadi et al. Thyroid. DOI: 10.1177/10507256251414948	No public full text located.
PMID 40488249 2026 Jan-Feb (online 2025 Jun 9)*	Persistent Palmar Rash Associated with Teprotumumab Treatment. Lance R He et al. Ophthalmic Plast Reconstr Surg. DOI: 10.1097/IOP.0000000000002985	No public full text located.

Full-text review - executive disposition

All seven publicly accessible publications were reviewed and reconciled with the initial evaluator. Four publications contain confirmed or potential patient-level ICSR information; the remaining three contribute special-population, durability, or therapeutic-context evidence.

1	42688166	No Tepezza ICSR; retain as special-population and benefit-risk evidence.	Agreed, with refinement: contextual evidence rather than a safety signal.
2	42438806	Valid expected hyperglycemia ICSR; likely non-serious.	Agreed; add a likely non-serious classification.
3	42386595	Valid serious expected hearing-loss ICSR.	Agreed; add likely seriousness due to persistent significant impairment.
4	41924363	Valid serious ICSR; expectedness requires RSI-specific assessment.	Partly agreed: hospitalization makes the case serious, while expectedness is not straightforward.
5	41487291	No conventional adverse-reaction ICSR; retain as possible lack-of-efficacy/durability observation.	Agreed, with a lack-of-efficacy and disease-recurrence qualifier.
6	41441522	No Tepezza ICSR; retain as unusual-etiology effectiveness evidence.	Agreed on non-ICSR status; revise from signal detection to contextual evidence.
7	40876911	At least three patient-linked potential ICSRs plus aggregate observations; regional rules apply.	Revised: the article is not solely non-ICSR signal material.

Case-counting and routing note

The first three conventional case publications meeting minimum criteria are PMIDs 42438806, 42386595, and 41924363. PMID 40876911 contains at least three patient-number-linked safety narratives (Patients 4, 5, and 6) that warrant case-processing assessment and duplicate search; other adverse effects in that paper are reported only in aggregate and require patient linkage or follow-up. Because PMID 40876911 is a retrospective non-interventional study, regulatory ICSR submission can vary by region.

Completed full-text medical reviews

D/R = dechallenge/rechallenge. Each assessment below preserves reported facts, separates author and reviewer causality, and avoids imputing unreported outcomes.

1. Case Report: Teprotumumab treatment in an HIV-infected patient with refractory thyroid eye disease: clinical and quantitative MRI evidence.

Citation / source	Mingyue Zheng et al. <i>Case Report: Teprotumumab treatment in an HIV-infected patient with refractory thyroid eye disease: clinical and quantitative MRI evidence</i> . Front Immunol. 2026 Aug 19. DOI: 10.3389/fimmu.2026.1911723. PMC open access
Patient / reporter	One identifiable 37-year-old man with HIV, active smoking, and refractory thyroid eye disease (TED). Publication authors are identifiable reporters.
Exposure	Teprotumumab: three infusions (500 mg, then 1,000 mg twice) from a planned eight; stopped because of financial/access constraints, not toxicity. Antiretroviral therapy continued.
Events / outcome	No adverse reaction was reported. The authors documented no opportunistic infection or HIV deterioration, normal pure-tone audiometry and blood glucose, and no infusion reaction or other clinically evident treatment-related adverse event.
Chronology; D/R	Clinical activity, proptosis, diplopia, and MRI findings improved. Drug discontinuation was unrelated to an adverse event; dechallenge and rechallenge are not applicable.
Seriousness	Not applicable because no adverse event was identified.
Expectedness	Not applicable.
Causality / confounders	No suspected adverse reaction. The publication provides negative safety observations in an immunocompromised patient.
ICSR criteria / routing	Identifiable patient, reporter, and exposure are present, but the adverse-event criterion is absent. The four minimum criteria for a conventional ICSR are not met.
Aggregate relevance	Retain for special-population exposure, treatment access, effectiveness, and aggregate benefit-risk context. Do not label the publication itself as a detected safety signal.
Reviewer reconciliation	Agreed, with refinement: contextual evidence rather than a safety signal.

2. Thyroid eye disease with teprotumumab-related hyperglycemia associated with increased growth hormone secretion.

Citation / source	Ken Sugawa et al. <i>Thyroid eye disease with teprotumumab-related hyperglycemia associated with increased growth hormone secretion</i> . JCEM Case Rep. 2026 Aug (online 2026 Jul 13). DOI: 10.1210/jcemcr/luag186. PMC open access
Patient / reporter	One identifiable 60-year-old woman with pre-existing type 2 diabetes and overweight (BMI 28.2). Publication authors are identifiable reporters.
Exposure	Teprotumumab was initiated for TED. Background diabetes treatment included metformin 500 mg and anagliptin 200 mg; baseline fasting plasma glucose was 135 mg/dL and HbA1c 7.2%.
Events / outcome	Hyperglycemia worsened three weeks after the first infusion (229 mg/dL) and by week six reached fasting glucose 312 mg/dL and HbA1c 10.5%. Urine ketones were negative. A 6 kg weight decrease was also reported.
Chronology; D/R	Teprotumumab was temporarily interrupted; metformin was increased and intensive insulin/continuous glucose monitoring were started. After improvement, teprotumumab was resumed at the patient's request and all eight infusions were completed. Insulin was tapered and stopped nine weeks after the final infusion.
Seriousness	Probably non-serious on published information: no hospitalization, diabetic ketoacidosis, hyperosmolar state, or life-threatening outcome was reported. Marked clinical severity and insulin treatment do not automatically establish regulatory seriousness; medical judgement could still apply.
Expectedness	Expected under the November 2025 US prescribing information, which describes hyperglycemia and highlights increased risk in patients with pre-existing diabetes or impaired glucose tolerance.
Causality / confounders	Reasonable possibility supported by temporal association, improvement with treatment interruption/intensive glycemic management, and the authors' diagnosis. Pre-existing diabetes and limited insulin secretory capacity are strong risk factors, not exclusions of causality.
ICSR criteria / routing	Identifiable patient, identifiable reporter, suspect teprotumumab, and a specified adverse reaction are present. Process as a literature ICSR under the applicable company and regional procedure.
Aggregate relevance	Known-risk characterization: diabetic susceptibility, monitoring, management, treatment interruption, and outcome after resumption.
Reviewer reconciliation	Agreed; add a likely non-serious classification.

3. Teprotumumab-associated persistent unilateral hearing loss in dysthyroid optic neuropathy: a case report with review of the literature.

Citation / source	Yuji Hiromatsu et al. <i>Teprotumumab-associated persistent unilateral hearing loss in dysthyroid optic neuropathy: a case report with review of the literature</i> . <i>Endocr J</i> . 2026 Jul 1. DOI: 10.1507/endocrj.EJ26-0064. Free article - J-STAGE
Patient / reporter	One identifiable 55-year-old woman with Graves' disease, TED with dysthyroid optic neuropathy, recurrent prior sudden sensorineural hearing loss (SNHL), and smoking history.
Exposure	Eight teprotumumab infusions were completed. Baseline audiometry immediately before treatment was normal despite an episode of right-sided SNHL 17 months earlier.
Events / outcome	Aural fullness and sound reverberation began after infusion five. Mild bilateral SNHL was documented after the final infusion, followed by right-sided low- to mid-frequency thresholds worsening to approximately 90 dB and PTA4 75 dB HL.
Chronology; D/R	Oral prednisolone and adjunctive therapies did not reverse the deficit. Audiograms through 198 days after the last infusion showed no significant recovery. Treatment was completed, so a conventional dechallenge was not performed; rechallenge did not occur.
Seriousness	Likely serious under persistent or significant disability/incapacity because the severe unilateral deficit remained for more than six months. The pre-teprotumumab hospital admission was for optic neuropathy and is not the seriousness basis for the hearing event.
Expectedness	Expected under the November 2025 US prescribing information, which states that TEPEZZA may cause severe hearing impairment, including hearing loss that may be permanent.
Causality / confounders	Reasonable possibility supported by onset during exposure, objective progression, persistence, biologic plausibility, and author assessment. Prior SNHL and smoking are important susceptibility factors; sudden idiopathic SNHL and Meniere disease were considered but not fully excluded.
ICSR criteria / routing	All four minimum ICSR criteria are met. Process as a serious literature ICSR and perform duplicate review.
Aggregate relevance	Known-risk characterization: delayed progression, low-frequency pattern, irreversibility, pre-existing auditory vulnerability, monitoring, and risk-benefit decisions about early interruption.
Reviewer reconciliation	Agreed; add likely seriousness due to persistent significant impairment.

4. Severe nausea and vomiting after teprotumumab initiation with corticosteroid pretreatment: a case report.

Citation / source	Justin Youn et al. <i>Severe nausea and vomiting after teprotumumab initiation with corticosteroid pretreatment: a case report</i> . Int J Ophthalmol. 2026 Apr 18. DOI: 10.18240/ijo.2026.04.25. PMC free full text
Patient / reporter	One identifiable 70-year-old woman with Graves' disease/TED, Hashimoto thyroiditis, hypertension, glaucoma, and baseline sensorineural hearing loss.
Exposure	Teprotumumab infusions every three weeks beginning March 2022, with 100 mg intravenous methylprednisolone pretreatment under the local infusion-center protocol.
Events / outcome	Episodic intractable nausea and vomiting began 2.5 weeks after the first infusion, accompanied by muscle spasms and profound weight loss from 78.8 to 46.3 kg (41.2%). Symptoms persisted for months after treatment.
Chronology; D/R	The patient continued teprotumumab because TED improved. She had one emergency-department visit during treatment, two afterward, and three hospitalizations during the six months after treatment. Multiple antiemetics had limited benefit. Symptoms and weight later improved after empiric glucocorticoid replacement.
Seriousness	Serious because the nausea/vomiting syndrome required inpatient hospitalization. 'Severe' describes intensity; 'serious' is the regulatory classification.
Expectedness	Nausea and weight decreased are listed in the November 2025 US prescribing information, but vomiting is not a listed common adverse reaction and the intractable, prolonged, repeatedly hospitalized presentation is more severe and specific than routine labeled nausea. Treat as potentially unexpected unless the applicable regional/company RSI supports expectedness.
Causality / confounders	A reasonable possibility remains and the authors retained teprotumumab in the differential. Major confounders include repeated methylprednisolone exposure, possible corticosteroid-withdrawal syndrome or tertiary adrenal insufficiency, thyroid abnormalities, and H. pylori. The diagnostic workup was incomplete, and improvement with steroid replacement is not a clean teprotumumab dechallenge.
ICSR criteria / routing	All four minimum ICSR criteria are met. Process as a serious literature ICSR, perform duplicate review, and document event-level expectedness against the controlling RSI.
Aggregate relevance	Characterizes severe gastrointestinal intolerance, substantial weight loss, hospitalization, prolonged course, and possible risk from routine corticosteroid pretreatment.
Reviewer reconciliation	Partly agreed: hospitalization makes the case serious, while expectedness is not straightforward.

5. Extraocular muscle volume reduction in a patient treated with two courses of teprotumumab for thyroid eye disease.

Citation / source	Hanna Richkind et al. <i>Extraocular muscle volume reduction in a patient treated with two courses of teprotumumab for thyroid eye disease</i> . Am J Ophthalmol Case Rep. 2026 Mar (online 2025 Dec 5). DOI: 10.1016/j.ajoc.2025.102490. PMC open access
Patient / reporter	One identifiable 84-year-old man with bilateral active TED and persistent ocular findings after corticosteroids.
Exposure	Two complete courses of eight teprotumumab infusions, separated by disease reactivation and additional corticosteroid treatment.
Events / outcome	The first course was explicitly completed without side effects. No teprotumumab adverse reaction was reported for the second course. TED symptoms reactivated seven months after the first course and flared again six months after the second.
Chronology; D/R	Both courses produced clinical and MRI improvement. Recurrences occurred after treatment completion; the authors framed these as TED progression/reactivation and durability limitations rather than adverse reactions.
Seriousness	No treatment-related adverse event was available for seriousness classification.
Expectedness	Not applicable to an adverse reaction. Disease recurrence may be handled as lack of efficacy/lack of sustained effect depending on procedure.
Causality / confounders	No suspected teprotumumab adverse reaction. The recurrent symptoms reflect underlying disease activity and incomplete durability of response.
ICSR criteria / routing	No conventional adverse-reaction ICSR. Under ICH E2D(R1), lack of efficacy without an associated adverse reaction is an 'other observation' whose ICSR submission depends on regional requirements or other authority conditions.
Aggregate relevance	Retain for durability, retreatment, disease recurrence, MRI response, and possible lack-of-efficacy evaluation.
Reviewer reconciliation	Agreed, with a lack-of-efficacy and disease-recurrence qualifier.

6. Case Reports of Teprotumumab as Treatment for Monoclonal Antibody-Induced Thyroid Orbitopathy.

Citation / source	Jared Moon et al. <i>Case Reports of Teprotumumab as Treatment for Monoclonal Antibody-Induced Thyroid Orbitopathy</i> . Reports (MDPI). 2025 Nov 26. DOI: 10.3390/reports8040246. PMC open access
Patient / reporter	Two identifiable patients: a 73-year-old woman with nivolumab-associated orbitopathy and a 36-year-old man with alemtuzumab-associated orbitopathy.
Exposure	Each patient completed eight teprotumumab infusions after inadequate response to intravenous methylprednisolone.
Events / outcome	Both patients had resolution of diplopia and substantial improvement in proptosis, motility, alignment, and imaging. No adverse event caused by teprotumumab was reported.
Chronology; D/R	The suspected drug-related disorders preceded teprotumumab and were attributed to nivolumab and alemtuzumab. Teprotumumab functioned as treatment, not the suspected cause.
Seriousness	Not applicable for Tepezza because no treatment-emergent adverse reaction was identified.
Expectedness	Not applicable.
Causality / confounders	No suspected Tepezza adverse reaction. The publication instead supports causal consideration of the antecedent monoclonal antibodies for TED.
ICSR criteria / routing	The adverse-event criterion for a Tepezza ICSR is absent. The current US indication broadly covers TED, so the unusual cause of TED should not automatically be labeled off-label Tepezza use.
Aggregate relevance	Retain for treatment effectiveness and use in rare secondary autoimmune TED. It does not itself constitute a Tepezza safety signal.
Reviewer reconciliation	Agreed on non-ICSR status; revise from signal detection to contextual evidence.

7. Teprotumumab Treatment in Patients with Steroid and Surgery-Resistant Dysthyroid Optic Neuropathy: A Case Series.

Citation / source	Anna Lucia Carretti et al. <i>Teprotumumab Treatment in Patients with Steroid and Surgery-Resistant Dysthyroid Optic Neuropathy: A Case Series</i> . Thyroid. 2025 Oct (online 2025 Aug 28)*. DOI: 10.1177/10507256251372194. PubMed-designated free article
Patient / reporter	Retrospective French case series of six numbered patients with steroid- and surgery-resistant dysthyroid optic neuropathy. Patient numbers can establish patient identifiability; authors are identifiable reporters.
Exposure	Teprotumumab was administered every three weeks under temporary French authorization. Four patients completed eight infusions; Patient 4 stopped after six and Patient 6 after four because of adverse effects.
Events / outcome	Patient 4: disabling tinnitus leading to discontinuation. Patient 6: diarrhea leading to discontinuation. Patient 5: worsening pre-existing diabetes (HbA1c 5.8% to 7.1%). Other aggregate reports included alopecia (3/6), gastrointestinal discomfort (2/6), new-onset hyperglycemia (1/6), and hearing loss (1/6), but these were not all linked to numbered patients.
Chronology; D/R	The publication directly links tinnitus and diarrhea to Patients 4 and 6 and links worsening diabetes to Patient 5. Details on onset, resolution, dechallenge, and rechallenge are limited. Inflammatory relapse in Patients 5 and 6 occurred months after treatment and was reported separately from adverse effects.
Seriousness	No death, hospitalization, or other explicit seriousness criterion was reported for the patient-linked events. They appear non-serious on available information; discontinuation and clinical intensity alone do not establish regulatory seriousness. Follow-up is warranted, particularly for the disabling tinnitus.
Expectedness	Tinnitus/hearing impairment, diarrhea, hyperglycemia, and alopecia are consistent with the known TEPEZZA safety profile in the November 2025 US prescribing information. Final coding and expectedness remain RSI- and event-specific.
Causality / confounders	The authors presented the events as teprotumumab adverse effects and treatment was stopped in Patients 4 and 6, supporting at least a reasonable possibility. The retrospective design and limited individual narratives constrain assessment.
ICSR criteria / routing	At minimum, Patients 4, 5, and 6 contain patient-linked safety information sufficient for case-processing assessment and duplicate search. Aggregate events not tied to a specific patient should be recorded for patient linkage rather than automatically entered as separate ICSRs. Because the paper reports a non-interventional retrospective study, regulatory ICSR submission may be excluded in some regions and must follow local requirements/company SOP.
Aggregate relevance	Retain the entire publication for aggregate tolerability, known-risk characterization, treatment discontinuation, and effectiveness in refractory optic neuropathy. Consider possible prior reporting through the temporary-access program during duplicate review.
Reviewer reconciliation	Revised: the article is not solely non-ICSR signal material.

Core surveillance log - included records

The following entries preserve the complete title/abstract screening log. The seven publicly available records incorporate the completed full-text dispositions above; the remaining twelve retain preliminary assessments pending full-text retrieval.

1. Adjunctive Orbital Radiation Therapy with Teprotumumab in Recalcitrant Thyroid Eye Disease: A Case Report.

Citation	Mannat Kaur et al. <i>Adjunctive Orbital Radiation Therapy with Teprotumumab in Recalcitrant Thyroid Eye Disease: A Case Report</i> . <i>Ophthalmic Plast Reconstr Surg</i> . 2026 Sep 11. DOI: 10.1097/IOP.0000000000003287.
Screening	PMID 42727021 ; duplicate: no by PMID; in scope: yes; full text: No public full text located; exclusion reason: not applicable.
Case relevance	One patient with recalcitrant, muscle-predominant TED and repeated reactivation after multiple teprotumumab courses; orbital radiation produced sustained control. Reporter: publication authors. Suspect product/exposure: teprotumumab. Minimum case criteria: preliminary only; confirm patient identifiability and event-level detail in full text.
Safety assessment	Vision-threatening disease course and possible lack-of-effect/relapse context. Seriousness and reportability require full text. Expectedness: Not a straightforward labeled adverse reaction; evaluate as disease progression or lack of effect. Causality: not finalized from title/abstract alone. Special situation: assess treatment failure, retreatment, off-label use, or special population where applicable.
Aggregate relevance	Durability, retreatment, adjunctive radiation, compressive optic neuropathy.
Disposition / QC	Potential individual case / aggregate relevance - full-text review required. Initial title/abstract decision complete 12 Sep 2026; full-text review and independent second review pending.

2. Teprotumumab for thyroid eye disease-associated superior ophthalmic vein periphlebitis: a case series.

Citation	Muhammad Abumanhal et al. <i>Teprotumumab for thyroid eye disease-associated superior ophthalmic vein periphlebitis: a case series</i> . <i>Graefes Arch Clin Exp Ophthalmol</i> . 2026 Sep 7. DOI: 10.1007/s00417-026-07491-y.
Screening	PMID 42702683 ; duplicate: no by PMID; in scope: yes; full text: No public full text located; exclusion reason: not applicable.
Case relevance	Seven patients with TED-associated superior ophthalmic vein periphlebitis treated with teprotumumab; radiologic resolution reported. Reporter: publication authors. Suspect product/exposure: teprotumumab. Minimum case criteria: preliminary only; confirm patient identifiability and event-level detail in full text.
Safety assessment	The periphlebitis preceded treatment and is not presented as a treatment-emergent event. Expectedness: Not applicable at abstract stage; this is primarily a therapeutic response series. Causality: not finalized from title/abstract alone. Special situation: assess treatment failure, retreatment, off-label use, or special population where applicable.
Aggregate relevance	Novel imaging phenotype and treatment response.
Disposition / QC	Aggregate-only therapeutic evidence; assess individual adverse events in full text if obtained. Initial title/abstract decision complete 12 Sep 2026; full-text review and independent second review pending.

3. Relapse Versus Rebound in Thyroid Eye Disease After Teprotumumab Treatment: A Retrospective Case Series.

Citation	Julie M Shabto et al. <i>Relapse Versus Rebound in Thyroid Eye Disease After Teprotumumab Treatment: A Retrospective Case Series</i> . Ophthalmic Plast Reconstr Surg. 2026 Sep-Oct (online 2026 Feb 20). DOI: 10.1097/IOP.0000000000003192.
Screening	PMID 41718601 ; duplicate: no by PMID; in scope: yes; full text: No public full text located; exclusion reason: not applicable.
Case relevance	Retrospective series of 26 patients with symptom return after teprotumumab; one active-phase patient developed new optic neuropathy. Reporter: publication authors. Suspect product/exposure: teprotumumab. Minimum case criteria: preliminary only; confirm patient identifiability and event-level detail in full text.
Safety assessment	Relapse/rebound and possible lack of effect; at least one medically important outcome is described. Expectedness: Not an adverse-reaction expectedness question until individual narratives and temporal details are reviewed. Causality: not finalized from title/abstract alone. Special situation: assess treatment failure, retreatment, off-label use, or special population where applicable.
Aggregate relevance	Durability and phase-specific relapse characterization.
Disposition / QC	Aggregate relevance with possible individual cases - full-text review required. Initial title/abstract decision complete 12 Sep 2026; full-text review and independent second review pending.

4. Case Report: Teprotumumab treatment in an HIV-infected patient with refractory thyroid eye disease: clinical and quantitative MRI evidence.

Citation	Mingyue Zheng et al. <i>Case Report: Teprotumumab treatment in an HIV-infected patient with refractory thyroid eye disease: clinical and quantitative MRI evidence</i> . Front Immunol. 2026 Aug 19. DOI: 10.3389/fimmu.2026.1911723.
Screening	PMID 42688166 ; duplicate: no by PMID; in scope: yes; full text: PMC open access; full-text review: complete.
Case relevance	One identifiable 37-year-old man with HIV, active smoking, and refractory thyroid eye disease (TED). Publication authors are identifiable reporters. No adverse reaction was reported. The authors documented no opportunistic infection or HIV deterioration, normal pure-tone audiometry and blood glucose, and no infusion reaction or other clinically evident treatment-related adverse event.
Safety assessment	Seriousness: Not applicable because no adverse event was identified. Expectedness: Not applicable. Causality: No suspected adverse reaction. The publication provides negative safety observations in an immunocompromised patient.
Aggregate relevance	Retain for special-population exposure, treatment access, effectiveness, and aggregate benefit-risk context. Do not label the publication itself as a detected safety signal.
Disposition / QC	No Tepezza ICSR; retain as special-population and benefit-risk evidence. Full-text assessment reconciled 12 Sep 2026; independent second review pending.

5. Thyroid eye disease with teprotumumab-related hyperglycemia associated with increased growth hormone secretion.

Citation	Ken Sugawa et al. <i>Thyroid eye disease with teprotumumab-related hyperglycemia associated with increased growth hormone secretion</i> . JCEM Case Rep. 2026 Aug (online 2026 Jul 13). DOI: 10.1210/jcemcr/luag186.
Screening	PMID 42438806 ; duplicate: no by PMID; in scope: yes; full text: PMC open access; full-text review: complete.
Case relevance	One identifiable 60-year-old woman with pre-existing type 2 diabetes and overweight (BMI 28.2). Publication authors are identifiable reporters. Hyperglycemia worsened three weeks after the first infusion (229 mg/dL) and by week six reached fasting glucose 312 mg/dL and HbA1c 10.5%. Urine ketones were negative. A 6 kg weight decrease was also reported.
Safety assessment	Seriousness: Probably non-serious on published information: no hospitalization, diabetic ketoacidosis, hyperosmolar state, or life-threatening outcome was reported. Marked clinical severity and insulin treatment do not automatically establish regulatory seriousness; medical judgement could still apply. Expectedness: Expected under the November 2025 US prescribing information, which describes hyperglycemia and highlights increased risk in patients with pre-existing diabetes or impaired glucose tolerance. Causality: Reasonable possibility supported by temporal association, improvement with treatment interruption/intensive glycemic management, and the authors' diagnosis. Pre-existing diabetes and limited insulin secretory capacity are strong risk factors, not exclusions of causality.
Aggregate relevance	Known-risk characterization: diabetic susceptibility, monitoring, management, treatment interruption, and outcome after resumption.
Disposition / QC	Valid expected hyperglycemia ICSR; likely non-serious. Full-text assessment reconciled 12 Sep 2026; independent second review pending.

6. Testosterone Dysregulation and Erectile Dysfunction Associated With Teprotumumab Infusions: A Case Report.

Citation	Sri Meghana Konda et al. <i>Testosterone Dysregulation and Erectile Dysfunction Associated With Teprotumumab Infusions: A Case Report</i> . Ophthalmic Plast Reconstr Surg. 2026 Jul 20. DOI: 10.1097/OP.0000000000003250.
Screening	PMID 42475232 ; duplicate: no by PMID; in scope: yes; full text: No public full text located; exclusion reason: not applicable.
Case relevance	One 52-year-old man had recurrent low testosterone, erectile dysfunction, and low libido during three teprotumumab courses, with improvement after treatment completion or replacement therapy. Reporter: publication authors. Suspect product/exposure: teprotumumab. Minimum case criteria: preliminary only; confirm patient identifiability and event-level detail in full text.
Safety assessment	Positive recurrence on re-exposure is described; seriousness is not established in the abstract. Expectedness: Potentially unlabeled endocrine/reproductive event; verify against current USPI. Causality: not finalized from title/abstract alone. Special situation: assess treatment failure, retreatment, off-label use, or special population where applicable.
Aggregate relevance	Rechallenge pattern and possible endocrine mechanism.
Disposition / QC	Potential ICSR candidate and signal-relevant case - full-text review required. Initial title/abstract decision complete 12 Sep 2026; full-text review and independent second review pending.

7. Teprotumumab-associated persistent unilateral hearing loss in dysthyroid optic neuropathy: a case report with review of the literature.

Citation	Yuji Hiromatsu et al. <i>Teprotumumab-associated persistent unilateral hearing loss in dysthyroid optic neuropathy: a case report with review of the literature</i> . <i>Endocr J</i> . 2026 Jul 1. DOI: 10.1507/endocrj.EJ26-0064.
Screening	PMID 42386595 ; duplicate: no by PMID; in scope: yes; full text: Free article - J-STAGE; full-text review: complete.
Case relevance	One identifiable 55-year-old woman with Graves' disease, TED with dysthyroid optic neuropathy, recurrent prior sudden sensorineural hearing loss (SNHL), and smoking history. Aural fullness and sound reverberation began after infusion five. Mild bilateral SNHL was documented after the final infusion, followed by right-sided low- to mid-frequency thresholds worsening to approximately 90 dB and PTA4 75 dB HL.
Safety assessment	Seriousness: Likely serious under persistent or significant disability/incapacity because the severe unilateral deficit remained for more than six months. The pre-teprotumumab hospital admission was for optic neuropathy and is not the seriousness basis for the hearing event. Expectedness: Expected under the November 2025 US prescribing information, which states that TEPEZZA may cause severe hearing impairment, including hearing loss that may be permanent. Causality: Reasonable possibility supported by onset during exposure, objective progression, persistence, biologic plausibility, and author assessment. Prior SNHL and smoking are important susceptibility factors; sudden idiopathic SNHL and Meniere disease were considered but not fully excluded.
Aggregate relevance	Known-risk characterization: delayed progression, low-frequency pattern, irreversibility, pre-existing auditory vulnerability, monitoring, and risk-benefit decisions about early interruption.
Disposition / QC	Valid serious expected hearing-loss ICSR. Full-text assessment reconciled 12 Sep 2026; independent second review pending.

8. Transformation of Thyroid Eye Disease Phenotype Following Teprotumumab Treatment.

Citation	Julie M Shabto et al. <i>Transformation of Thyroid Eye Disease Phenotype Following Teprotumumab Treatment</i> . <i>Ophthalmic Plast Reconstr Surg</i> . 2026 Jul-Aug (online 2025 Nov 6). DOI: 10.1097/IOP.0000000000003110.
Screening	PMID 41196136 ; duplicate: no by PMID; in scope: yes; full text: No public full text located; exclusion reason: not applicable.
Case relevance	One 35-year-old woman had reactivation and a shift to severe muscle-predominant TED after treatment, with compressive optic neuropathy requiring decompression. Reporter: publication authors. Suspect product/exposure: teprotumumab. Minimum case criteria: preliminary only; confirm patient identifiability and event-level detail in full text.
Safety assessment	Serious vision-threatening disease progression; potential lack-of-effect or post-treatment worsening. Expectedness: Not a straightforward labeled reaction; assess disease natural history and alternative causes. Causality: not finalized from title/abstract alone. Special situation: assess treatment failure, retreatment, off-label use, or special population where applicable.
Aggregate relevance	Phenotypic change, durability, and retreatment implications.
Disposition / QC	Potential individual case / aggregate relevance - full-text review required. Initial title/abstract decision complete 12 Sep 2026; full-text review and independent second review pending.

9. Recalcitrant thyroid eye disease responsive to targeted EGFR inhibition in a patient with lung adenocarcinoma: a case report.

Citation	Rachel E Cain et al. <i>Recalcitrant thyroid eye disease responsive to targeted EGFR inhibition in a patient with lung adenocarcinoma: a case report</i> . Orbit. 2026 Jun (online 2026 Mar 12). DOI: 10.1080/01676830.2026.2632696.
Screening	PMID 41818461 ; duplicate: no by PMID; in scope: yes; full text: No public full text located; exclusion reason: not applicable.
Case relevance	One patient with refractory TED had previously received teprotumumab; the report focuses on improvement with osimertinib. Reporter: publication authors. Suspect product/exposure: teprotumumab. Minimum case criteria: preliminary only; confirm patient identifiability and event-level detail in full text.
Safety assessment	Teprotumumab is background exposure, not the suspected cause of the reported response. Expectedness: Not applicable for Tepezza based on the abstract. Causality: not finalized from title/abstract alone. Special situation: assess treatment failure, retreatment, off-label use, or special population where applicable.
Aggregate relevance	Context on refractory disease and alternative targets.
Disposition / QC	Contextual/noncase safety information for Tepezza; retain for traceability. Initial title/abstract decision complete 12 Sep 2026; full-text review and independent second review pending.

10. Variability Between Changes in Ocular Alignment and Gorman Diplopia Score After Teprotumumab in Thyroid Eye Disease.

Citation	Madhura A Tamhankar et al. <i>Variability Between Changes in Ocular Alignment and Gorman Diplopia Score After Teprotumumab in Thyroid Eye Disease</i> . Neuroophthalmology. 2026 May 18. DOI: 10.1080/01658107.2026.2673534.
Screening	PMID 42694662 ; duplicate: no by PMID; in scope: yes; full text: No public full text located; PMC embargo to 18 May 2027; exclusion reason: not applicable.
Case relevance	Retrospective series of 17 patients with TED-related diplopia treated with teprotumumab; subjective and objective outcomes diverged in seven. Reporter: publication authors. Suspect product/exposure: teprotumumab. Minimum case criteria: preliminary only; confirm patient identifiability and event-level detail in full text.
Safety assessment	No individual adverse reaction is described in the abstract. Expectedness: Not applicable; efficacy-measurement study. Causality: not finalized from title/abstract alone. Special situation: assess treatment failure, retreatment, off-label use, or special population where applicable.
Aggregate relevance	Outcome measurement and benefit characterization.
Disposition / QC	Aggregate-only evidence; PMC full text is embargoed until 2027-05-18. Initial title/abstract decision complete 12 Sep 2026; full-text review and independent second review pending.

11. From orbit to cortex: recognising teprotumumab-induced encephalopathy.

Citation	Alan Spicer et al. <i>From orbit to cortex: recognising teprotumumab-induced encephalopathy</i> . BMJ Case Rep. 2026 May 8. DOI: 10.1136/bcr-2025-271752.
Screening	PMID 42103337 ; duplicate: no by PMID; in scope: yes; full text: No public full text located; exclusion reason: not applicable.
Case relevance	One man in his early 60s developed neuropsychiatric changes after starting teprotumumab and was treated with plasma exchange. Reporter: publication authors. Suspect product/exposure: teprotumumab. Minimum case criteria: preliminary only; confirm patient identifiability and event-level detail in full text.
Safety assessment	Teprotumumab-induced encephalopathy is the authors' diagnosis; serious/medically important event likely. Expectedness: Potentially unlabeled; encephalopathy has also been monitored as a safety-signal topic. Causality: not finalized from title/abstract alone. Special situation: assess treatment failure, retreatment, off-label use, or special population where applicable.
Aggregate relevance	Rare neurologic risk and management.
Disposition / QC	Potential serious ICSR candidate and signal-relevant case - full-text review required. Initial title/abstract decision complete 12 Sep 2026; full-text review and independent second review pending.

12. Severe nausea and vomiting after teprotumumab initiation with corticosteroid pretreatment: a case report.

Citation	Justin Youn et al. <i>Severe nausea and vomiting after teprotumumab initiation with corticosteroid pretreatment: a case report</i> . Int J Ophthalmol. 2026 Apr 18. DOI: 10.18240/ijo.2026.04.25.
Screening	PMID 41924363 ; duplicate: no by PMID; in scope: yes; full text: PMC free full text; full-text review: complete.
Case relevance	One identifiable 70-year-old woman with Graves' disease/TED, Hashimoto thyroiditis, hypertension, glaucoma, and baseline sensorineural hearing loss. Episodic intractable nausea and vomiting began 2.5 weeks after the first infusion, accompanied by muscle spasms and profound weight loss from 78.8 to 46.3 kg (41.2%). Symptoms persisted for months after treatment.
Safety assessment	Seriousness: Serious because the nausea/vomiting syndrome required inpatient hospitalization. 'Severe' describes intensity; 'serious' is the regulatory classification. Expectedness: Nausea and weight decreased are listed in the November 2025 US prescribing information, but vomiting is not a listed common adverse reaction and the intractable, prolonged, repeatedly hospitalized presentation is more severe and specific than routine labeled nausea. Treat as potentially unexpected unless the applicable regional/company RSI supports expectedness. Causality: A reasonable possibility remains and the authors retained teprotumumab in the differential. Major confounders include repeated methylprednisolone exposure, possible corticosteroid-withdrawal syndrome or tertiary adrenal insufficiency, thyroid abnormalities, and H. pylori. The diagnostic workup was incomplete, and improvement with steroid replacement is not a clean teprotumumab dechallenge.
Aggregate relevance	Characterizes severe gastrointestinal intolerance, substantial weight loss, hospitalization, prolonged course, and possible risk from routine corticosteroid pretreatment.
Disposition / QC	Valid serious ICSR; expectedness requires RSI-specific assessment. Full-text assessment reconciled 12 Sep 2026; independent second review pending.

13. N-acetylcysteine and Vitamin C oral antioxidant therapy for teprotumumab-related hearing dysfunction: case series and review of literature.

Citation	Elijah Gregory et al. <i>N-acetylcysteine and Vitamin C oral antioxidant therapy for teprotumumab-related hearing dysfunction: case series and review of literature</i> . Orbit. 2026 Apr (online 2026 Feb 17). DOI: 10.1080/01676830.2025.2608253.
Screening	PMID 41700562 ; duplicate: no by PMID; in scope: yes; full text: No public full text located; exclusion reason: not applicable.
Case relevance	Three patients developed ear fullness and hearing loss during infusions and improved after N-acetylcysteine plus vitamin C. Reporter: publication authors. Suspect product/exposure: teprotumumab. Minimum case criteria: preliminary only; confirm patient identifiability and event-level detail in full text.
Safety assessment	Treatment-emergent hearing dysfunction in three patients; individual seriousness is not established in the abstract. Expectedness: Preliminary label-aligned topic (hearing impairment); treatment response adds characterization. Causality: not finalized from title/abstract alone. Special situation: assess treatment failure, retreatment, off-label use, or special population where applicable.
Aggregate relevance	Management of ototoxicity and reversibility.
Disposition / QC	Potential multiple ICSRs / known-risk characterization - full-text review required. Initial title/abstract decision complete 12 Sep 2026; full-text review and independent second review pending.

14. Teprotumumab Use in Thyroid Eye Disease: Clinical Outcomes in the United Arab Emirates- a First Regional Case Series.

Citation	H Eatamadi et al. <i>Teprotumumab Use in Thyroid Eye Disease: Clinical Outcomes in the United Arab Emirates- a First Regional Case Series</i> . Semin Ophthalmol. 2026 Apr (online 2025 May 21)*. DOI: 10.1080/08820538.2025.2503911.
Screening	PMID 40396469 ; duplicate: no by PMID; in scope: yes; full text: No public full text located; exclusion reason: not applicable.
Case relevance	Retrospective series of 14 patients (25 eyes) in the UAE completing eight cycles; efficacy and tolerable adverse effects were reported. Reporter: publication authors. Suspect product/exposure: teprotumumab. Minimum case criteria: preliminary only; confirm patient identifiability and event-level detail in full text.
Safety assessment	Adverse effects are not individually described in the abstract. Expectedness: Pending individual event details. Causality: not finalized from title/abstract alone. Special situation: assess treatment failure, retreatment, off-label use, or special population where applicable.
Aggregate relevance	Regional effectiveness, durability, and safety experience.
Disposition / QC	Aggregate-only evidence; individual case assessment requires full text. Initial title/abstract decision complete 12 Sep 2026; full-text review and independent second review pending.

15. Extraocular muscle volume reduction in a patient treated with two courses of teprotumumab for thyroid eye disease.

Citation	Hanna Richkind et al. <i>Extraocular muscle volume reduction in a patient treated with two courses of teprotumumab for thyroid eye disease.</i> Am J Ophthalmol Case Rep. 2026 Mar (online 2025 Dec 5). DOI: 10.1016/j.ajoc.2025.102490.
Screening	PMID 41487291 ; duplicate: no by PMID; in scope: yes; full text: PMC open access; full-text review: complete.
Case relevance	One identifiable 84-year-old man with bilateral active TED and persistent ocular findings after corticosteroids. The first course was explicitly completed without side effects. No teprotumumab adverse reaction was reported for the second course. TED symptoms reactivated seven months after the first course and flared again six months after the second.
Safety assessment	Seriousness: No treatment-related adverse event was available for seriousness classification. Expectedness: Not applicable to an adverse reaction. Disease recurrence may be handled as lack of efficacy/lack of sustained effect depending on procedure. Causality: No suspected teprotumumab adverse reaction. The recurrent symptoms reflect underlying disease activity and incomplete durability of response.
Aggregate relevance	Retain for durability, retreatment, disease recurrence, MRI response, and possible lack-of-efficacy evaluation.
Disposition / QC	No conventional adverse-reaction ICSR; retain as possible lack-of-efficacy/durability observation. Full-text assessment reconciled 12 Sep 2026; independent second review pending.

16. Beyond the Eyes: Is Teprotumumab Effective for Thyroid Dermopathy?

Citation	Dana Hamadi et al. <i>Beyond the Eyes: Is Teprotumumab Effective for Thyroid Dermopathy?</i> Thyroid. 2026 Feb (online 2026 Jan 13). DOI: 10.1177/10507256251414948.
Screening	PMID 41791882 ; duplicate: no by PMID; in scope: yes; full text: No public full text located; exclusion reason: not applicable.
Case relevance	Five patients with severe pretibial dermopathy and TED improved with teprotumumab; mild hearing loss, cramps, fatigue, and one severe GI event were reported. Reporter: publication authors. Suspect product/exposure: teprotumumab. Minimum case criteria: preliminary only; confirm patient identifiability and event-level detail in full text.
Safety assessment	Multiple treatment-emergent events; one event caused discontinuation. Individual case details are not in the abstract. Expectedness: Several topics appear label-aligned; severe GI event and exact terms require full-text review. Causality: not finalized from title/abstract alone. Special situation: assess treatment failure, retreatment, off-label use, or special population where applicable.
Aggregate relevance	Off-label dermopathy use, recurrence, retreatment, and tolerability.
Disposition / QC	Aggregate relevance with potential individual ICSRs - full-text review required. Initial title/abstract decision complete 12 Sep 2026; full-text review and independent second review pending.

17. Persistent Palmar Rash Associated with Teprotumumab Treatment.

Citation	Lance R He et al. <i>Persistent Palmar Rash Associated with Teprotumumab Treatment</i> . <i>Ophthalmic Plast Reconstr Surg</i> . 2026 Jan-Feb (online 2025 Jun 9)*. DOI: 10.1097/IOP.0000000000002985.
Screening	PMID 40488249 ; duplicate: no by PMID; in scope: yes; full text: No public full text located; exclusion reason: not applicable.
Case relevance	One 77-year-old man developed bilateral palmar rash during treatment that persisted after completion. Reporter: publication authors. Suspect product/exposure: teprotumumab. Minimum case criteria: preliminary only; confirm patient identifiability and event-level detail in full text.
Safety assessment	Treatment-emergent persistent rash; seriousness is not established in the abstract. Expectedness: Specific palmar rash expectedness is unclear; verify against current label terminology. Causality: not finalized from title/abstract alone. Special situation: assess treatment failure, retreatment, off-label use, or special population where applicable.
Aggregate relevance	Persistence of cutaneous adverse effects.
Disposition / QC	Potential ICSR candidate - full-text review required. Initial title/abstract decision complete 12 Sep 2026; full-text review and independent second review pending.

18. Case Reports of Teprotumumab as Treatment for Monoclonal Antibody-Induced Thyroid Orbitopathy.

Citation	Jared Moon et al. <i>Case Reports of Teprotumumab as Treatment for Monoclonal Antibody-Induced Thyroid Orbitopathy</i> . <i>Reports (MDPI)</i> . 2025 Nov 26. DOI: 10.3390/reports8040246.
Screening	PMID 41441522 ; duplicate: no by PMID; in scope: yes; full text: PMC open access; full-text review: complete.
Case relevance	Two identifiable patients: a 73-year-old woman with nivolumab-associated orbitopathy and a 36-year-old man with alemtuzumab-associated orbitopathy. Both patients had resolution of diplopia and substantial improvement in proptosis, motility, alignment, and imaging. No adverse event caused by teprotumumab was reported.
Safety assessment	Seriousness: Not applicable for Tepezza because no treatment-emergent adverse reaction was identified. Expectedness: Not applicable. Causality: No suspected Tepezza adverse reaction. The publication instead supports causal consideration of the antecedent monoclonal antibodies for TED.
Aggregate relevance	Retain for treatment effectiveness and use in rare secondary autoimmune TED. It does not itself constitute a Tepezza safety signal.
Disposition / QC	No Tepezza ICSR; retain as unusual-etiology effectiveness evidence. Full-text assessment reconciled 12 Sep 2026; independent second review pending.

19. Teprotumumab Treatment in Patients with Steroid and Surgery-Resistant Dysthyroid Optic Neuropathy: A Case Series.

Citation	Anna Lucia Carretti et al. <i>Teprotumumab Treatment in Patients with Steroid and Surgery-Resistant Dysthyroid Optic Neuropathy: A Case Series</i> . Thyroid. 2025 Oct (online 2025 Aug 28)*. DOI: 10.1177/10507256251372194.
Screening	PMID 40876911 ; duplicate: no by PMID; in scope: yes; full text: PubMed-designated free article; full-text review: complete.
Case relevance	Retrospective French case series of six numbered patients with steroid- and surgery-resistant dysthyroid optic neuropathy. Patient numbers can establish patient identifiability; authors are identifiable reporters. Patient 4: disabling tinnitus leading to discontinuation. Patient 6: diarrhea leading to discontinuation. Patient 5: worsening pre-existing diabetes (HbA1c 5.8% to 7.1%). Other aggregate reports included alopecia (3/6), gastrointestinal discomfort (2/6), new-onset hyperglycemia (1/6), and hearing loss (1/6), but these were not all linked to numbered patients.
Safety assessment	Seriousness: No death, hospitalization, or other explicit seriousness criterion was reported for the patient-linked events. They appear non-serious on available information; discontinuation and clinical intensity alone do not establish regulatory seriousness. Follow-up is warranted, particularly for the disabling tinnitus. Expectedness: Tinnitus/hearing impairment, diarrhea, hyperglycemia, and alopecia are consistent with the known TEPEZZA safety profile in the November 2025 US prescribing information. Final coding and expectedness remain RSI- and event-specific. Causality: The authors presented the events as teprotumumab adverse effects and treatment was stopped in Patients 4 and 6, supporting at least a reasonable possibility. The retrospective design and limited individual narratives constrain assessment.
Aggregate relevance	Retain the entire publication for aggregate tolerability, known-risk characterization, treatment discontinuation, and effectiveness in refractory optic neuropathy. Consider possible prior reporting through the temporary-access program during duplicate review.
Disposition / QC	At least three patient-linked potential ICSRs plus aggregate observations; regional rules apply. Full-text assessment reconciled 12 Sep 2026; independent second review pending.

Excluded candidate records and notable out-of-scope evidence

Two records met the case-word candidate query but were excluded after abstract screening.

41783234	Pharmacologic Therapies for Active Moderate-to-Severe TED: A Comprehensive Systematic Review.	Systematic review; not an original patient-level case report or series.
41847275	Ocular Neuromyotonia in Thyroid Eye Disease.	Patient was not reported as exposed to teprotumumab; the drug appears only as a possible future therapy.

Notable aggregate-safety article outside the case-report list

[PMID 42706935](#) - *Drug-associated central nervous system vasculitis and reversible cerebral vasoconstriction syndrome: A global disproportionality analysis of the WHO pharmacovigilance database (VigiBase)*. This is not an individual case report, but the abstract reports a teprotumumab-related RCVS disproportionality signal based on six VigiBase cases. Retain it for a future aggregate-safety surveillance log.

Next workflow step

Document attempts to obtain the twelve unavailable full texts, update their preliminary assessments when obtained, and conduct an independent second review of screening and medical conclusions. For publications routed to case processing, confirm controlling regional requirements, reference safety information, duplicate status, MedDRA coding, and follow-up needs.

Limitations

PubMed indexing and public-access status can change. Recent records may not yet have complete publication-type indexing. Twelve records remain limited to title/abstract evidence. Case series may not contain sufficient individual-level detail, and case submission rules vary by region and company procedure. Expectedness conclusions use the stated US reference label for this educational exercise rather than a sponsor-specific Company Core Data Sheet. This independent portfolio analysis is not affiliated with FDA or a product manufacturer and is not a regulatory determination or medical advice.